ARI (PNEUMONIA) CLINICOSOCIAL CASE PRESENTATION

SWATHI KANNAN

7 TH TERM

BMCRI

DEMOGRAPHIC DETAILS

Name : Master ABC

Age : 3 years 1 month

Sex: Male

Religion : Hindu

Address-Bangarapet, Deshalli, KGF main road, Kolar

Informant: mother(Reliable): Age-25years Education – 4th std

DOA- 31/1/20

DOE-31/1/20

FAMILY STRUCTURE

Number of Family members :4, Type of family : Nuclear

Head of family- Father of ABC- XYZ

Total family income: 10000 per month

Percapita income : 2500 per month

Socioeconomic status : Upper lower class

Modified Kuppuswami Classification

Education	**2**
Monthly income	2
Occupation	3
Score	7
	Upper Lower class

S.No	Name	Gender	Age	Relation to head of Family	Marital status	Education	Occupation	Income	Health Remark
1	XYZ	M	30	---	Married	5th std	Labourer	5000	SMOKER (10years)
2	GHI	F	25	Wife	Married	4th std	Labourer	5000	CHEWS PAAN
3	JKL	F	5	Daughter	---	---	----	---	----
4	ABC	M	3 years 1 month	Son	---	---	----	---	----

CC-

1)Nasal stuffiness since 7 days

2) Cough and fever since 5 days

3) Fast breathing since 4 days

Patient was apparently normal 7 days ago when she developed nasal stuffiness and runny nose. It relieved with medications .

The child developed wet cough 5 days ago which was insidious in onset, not associated with post-tussive vomiting. The cough is gradually progressive in nature . It has now begun to interfere with the child's sleep .There was no diurnal variation of cough.

Patient developed fever 5 days ago. It was high grade in nature associated with rigors. It was not associated with rashes. The fever was intermittent in nature relieved with medications. Child was well during the inter febrile period. Older sibling had fever 10 days ago.

There is history of fast breathing since 4 days. It was insidious in onset. It was associated with difficulty in breathing and noisy breathing.

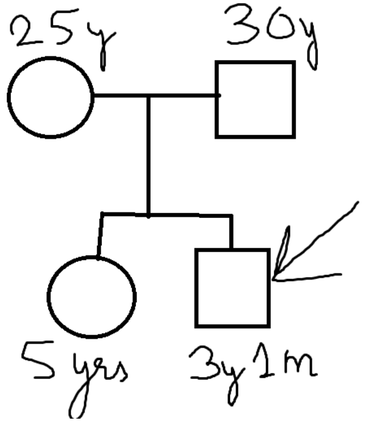
No H/O ear tugging , ear discharge
No H/O mouth breathing, voice change, snoring
No H/O wheeze

No H/O convulsions or vomiting
No H/O unconsciousness
No H/O diarrhoea
No H/O bluish discolouration of lips or extremities
No H/O refusal to feed

No H/O repeated hospitalizations or nebulisations
No H/O chronic cough in the family or in neighbours.

FAMILY HISTORY

No H/O genetic or chromosomal disorders in the family
No H/O Bronchial asthma in the family
Non consanguineous family



PAST HISTORY

The child had h/o diarrhoea 6 months back.
There is no h/o suck rest suck cycle. No h/o sweating of forehead after feeds.

PREVIOUS PREGNANACY

First child (female)- 5years
Normal vaginal delivery
No contraceptives used after birth of the child.
Inter-pregnancy interval – 1 year 11months

BIRTH HISTORY

ANTENATAL HISTORY

Booked Pregnancy
Iron and Folic acid tablets were taken.
In first month dT booster was taken
No H/O fever or rashes during the pregnancy.
No H/O increased blood sugar levels and no H/O increased BP during pregnancy.
All 3 scans were done

NATAL HISTORY

Term delivery normal vaginal delivery at hospital.
Birth weight- 2.4 kg, cried immediately after birth
Breastfeeding was started within 30 min of delivery.

POSTNATAL HISTORY

Child was exclusively breastfed up to 6 months of age.
Ragi-sori was started as complementary feeds during weaning.
After 1 year of age child began sharing the family pot .
No H/O Hospital admissions
Mother underwent IUCD placement within 1 day of delivery.

IMMUNIZATION HISTORY

AGE	SCHEDULE
At birth	BCG , OPV-0 , HepB-0
6weeks	Penta-1 , Rota-1 , fIPV-1 , OPV-1
10weeks	Penta-2, Rota -2, OPV-2
14weeks	Penta-3 , Rota-3 , fIPV-2 , OPV-3
9months	Measles and Rubella , Vitamin A
18 months	DPT booster, OPV booster, Measles and Rubella-2nd dose

Immunisation is complete according to NIS.

DEVELOPMENTAL HISTORY

GROSS	**RIDES TRICYCLE, ALTERNATE FEET GOING UPSATIRS**
FINE	TOWER OF 9 BLOCKS , COPIES CIRCLE
SOCIAL AND ADAPTIVE	Shares toys, knows full name and gender
LANGUAGE	Asks questions , knows full name and gender

For the present age of 3 years 1 month the latest developmental milestones are achieved in all 4 domains. So development is appropriate for age.

PERSONAL HYGIENE STATUS- Mother

S.No	Component	Condition	Remarks
1	Hair, Nails	clean	long nails,
2	Teeth	Poor	Chews paan
3	Bath	regular	
4	Washing hands 1)before and after feeding child 2)After ablution with soap and water	Done Only water	
5	Clothes	clean	
6	Footwear	good	slippers

DIET HISTORY

Diet is mixed predominant

Staple food : Rice

Total calories required-1060kcal

Total proteins required- 12.9 g

Calories consumed = 706kcal

Calories deficit=354kcal

Calorie Deficit percentage=33%

Protein consumed=11.53g

Protein deficit =1.37g

Protein deficit percentage =10.6%

MEALS	FOOD STUFF CONSUMED	CALORIES	PROTEIN
BREAKFAST	LEMON RICE(1cup)	110	2.2
11a.m	Milk (1/2 glass)	83kcal	4
LUNCH	Kitchri(1cup)	355kcal	3
5p.m	Biscuit(2 parle-G)	37kcal	0.33g
DINNER	Bread -2slices	121	2

ENVIRONMENTAL HISTORY

Pucca house.
Separate
4 members , enough space
Floor - Cement
Roof – Cement
Walls- Plastered
Rented house with 1 room , 1 hall
1 door in home only
one window in hall cum attached kitchen – window size 1/6th floor area
one window in room – window size 1/4th of floor area.

over crowding present

Lighting is inadequate – both natural and artificial

ventilation is inadequate

Attached kitchen, with cooking platform. Firewood is used.

Food grains are stored in air tight containers.

Cooked food is eaten on the same day it is prepared.

Drinking water- tap water boiled. Water for drinking is stored in pots .

A cup is used to retrieve the water.

One bathroom outside home with
continuous water supply. Proper Drainage system is seen
Surroundings- Dirty

Waste disposal : regularly collected(once a week) Occasionally waste is disposed to nearby vacant site.

No sites of stagnant water around the house .

Domestic Pets- Cat

Distance of nearest health facility : 5km

SUMMARY

Here is a 3 years 1 month old male child , who came to the hospital after mother noticed nasal stuffiness for 7 days, cough and high fever for 5 days , fast breathing for 4 days with flaring of ala-nasi, intercostal and subcostal retractions and grunting . It was associated with difficulty in breathing and noisy breathing . There is no past history of suck-rest-suck cycle , sweating of forehead after feeding or prolonged feeding . The child belongs to a upper lower class family according modified Kuppuswamy classification. He was born out of a non consanguineous marriage. His immunization history is up to date according NIS. His diet is deficient in calories, protein ,fruits and green leafy vegetables.

GENERAL PHYSICAL EXAMINATION

The child is alert but quiet.
He was examined in supine position.

VITALS-
Temp- 38 degress Celsius , measured in axilla(Normal values= 36.5 to 37.5 degrees celsius)

Pulse- 120bpm (Normal H.R= 100bpm) , normal in rhythm and volume.

Respiratory Rate- 50 breaths per minute,(NORMAL VALUES- 30 cycles per min)
abdomino-thoracic type (flaring of ala nasi and usage of accessory muscles of respiration was seen)

BP-100/60mmHg measured over right brachial artery in sitting position (Normal value= 76 to 96mmhg of systolic)
O2 saturation- 96%
NO STRIDOR IN CALM CHILD

HEAD TO TOE EXAMINATION

Skull shape- Normal, fontanels closed .
Eyes- not Sunken, no pallor, no Bitot's spots
Nose- Flaring of ala nasi, congested nasal mucosa
No deviated nasal septum
Lips- no cleft lip
no cyanosis
Tongue- appears moist
Oropharynx- Normal Tonsils
Posterior pharyngeal wall- inflamed
Palate- no cleft palate, no high arched palate
Teeth- normal dentition
Nails- no clubbing
Skin- normal, BCG scar over left arm
Spine and back – normal
No edema
No lymphadenopathy

ANTHROPOMETRY

	OBSERVED	EXPECTED	INTERPRETATION
WEIGHT	12.9	14.5	-1SD
HEIGHT	93cm	96.7cm	-1SD
WEIGHT FOR HEIGHT	0SD	0SD	
MAC	14cm	15cm	
HC	50cm	50cm	

RESPIRATORY SYSTEM EXAMINATION

INSPECTION

Shape of chest- Elliptical in shape
Spine- Normal
Trachea- appears central
Apex beat- not visible
Movements with respiration decreased on both sides.
Subcostal and intercostal retractions are seen. There is use of accessory muscles of respiration.

PALPATION

Inspectory findings were confirmed on palpation.
No tenderness, no local rise in temperature
Position of trachea central
Apical impulse felt- in left 4th intercostal space medial to midclavicular line

PERCUSSION

Decreased resonance on percussing both lungs

AUSCULTATION

Diffuse crepitation were heard in both the lungs.

SYSTEMIC EXAMINATION

Cardiovascular examination

S1 and S2 heard. No murmurs

Abdominal examination

No mass per abdomen , No Organomegaly

CNS examination

No neurological deficits present

Remarks and Clinical Diagnosis

Here is a 3 years 1 month old male child , who came to the hospital after mother noticed nasal stuffiness for 7 days, cough and high grade fever for 5 days , fast breathing for 4 days with difficulty in breathing and noisy breathing .
Examination revealed decreased chest movements both sides and use of accessory muscles of respiration . **Auscultation** revealed diffuse crepitation on both sides.
diagnosis- A case of viral bronchopneumonia, belonging to PINK (SEVERE PNEUMONIA) according to IMNCI.

KNOWLEDGE	ATTITUDE	PRACTICE
1)Causative factors- Ignorant about risk factors like overcrowding, smoking, firewood fuel. Lack of awareness about good nutrition	Careless	Poor practice

TREAMENT

According IMNCI, child should be treated with antibiotics intramuscular (Benzyl Penicillin/ Ampicillin/Chloramphenicol) for first 48 hours, given 6 hourly .If condition improves, switch to oral ampicillin, chloramphenicol or procaine penicillin. If condition does not improve CHANGE antibiotics.
Antibiotic therapy should be given for minimum of 5 days and continued for at least 3 days after the child gets well.

Monitor food and fluid intake

Advice mother on home management on discharge.